

CYP		Substrats	Inhibiteurs	Inducteurs
CYP1A2		Théophylline Certains antiépileptiques Halopéridol Caféine Certains antipsychotiques (Clozapine)	Ciprofloxacine Fluvoxamine Inflammation sévère (Pneumonie, ...) Arrêt du tabac	Tabac : Goudron
CYP2C	CYP2C19	IPP Clopidogrel (pro-drug) Antiépileptiques Indométacine Escitalopram	Antifongiques (azoles) Cimetidine Isoniazid IPP (e.g., omeprazole)	Carbamazepine Rifampicin Millepertuis Prednisolone
	CYP2C9	AINS Acénocoumarol (Sintrom) Antiépileptiques Antidiabétiques oraux (e.g., rosiglitazone) Angiotensin II antagonists	Sulfonamides (e.g., sulfamethoxazole) Sodium valproate Metronidazole Amiodarone Antifongiques (azoles) Chloramphénicol	Carbamazepine Rifampicin Millepertuis Griseofulvin
CYP2D6		Beta-bloquants (e.g., métoprolol) Antipsychotiques (e.g., halopéridol) Certains antidépresseurs : SSRI (e.g., fluoxetine, fluvoxamine), SNRI (e.g., venlafaxine), et Tricyclique (e.g., amitriptyline) Antiarythmiques (e.g., flécaïnide) Tamoxifène (pro-drug) Codéine (pro-drug)	Amiodarone Antipsychotiques and antidépresseurs (e.g., fluoxetine , paroxetine, duloxetine) Metoclopramide	Rifampicin Dexaméthasone
CYP2E1		OH	Disulfiram Ethanol (aigu)	Isoniazide OH (chronique) Nicotine
CYP3A4		Méthadone Statines (sauf pravastatine) Hormones stéroïdiennes (e.g., estrogènes, glucocorticoïdes, testostérone) Contraceptifs oraux Carbamazepine et autres antiépileptiques Haloperidol Immunosuppresseurs Cyclosporine Tacrolimus Macrolides Benzodiazépines Anticoagulants oraux (-xaban) AVK (Acénocoumarol (Sintrom), Phenprocoumon (Marcoumar)) Anti-HIV Anticalciques Itraconazole Codéine (pro-drug)	Amiodarone Macrolides (érythromycine, clarithromycine) Certains anti-HIV (e.g., ritonavir) Grapefruit juice Sodium valproate Ritonavir (Paxlovid) Cimétidine Chloramphénicol Antifongiques (azoles)	Carbamazepine Rifampicin Millepertuis Certains anti-HIV Griséofulvine Barbituriques (e.g., phénobarbital) Phénytoïne (aussi substrat) Glucocorticoïdes
Pgp		Indinavir Rivaroxaban	Ciclosporine	Rifampicine

Interactions Pharmacocinétiques : (modification de l'effet par modification de la concentration plasmatique (ADME))

- Absorption :
 - Interaction physico-chimique: ex: quinolones et antiacides ou cations
 - Inhibition/ induction de la Pgp
 - Inhibition/induction CYP450 intestinaux
 - Métabolisme : CYP p450
 - Inducteurs*: exemples : **rifampicine**, **carbamazépine** (antiépileptiques), **millepertuis**... → Chercher si manque d'effet
 - Inhibiteurs*: exemples : AB macrolides, « azolés », anti VIH, grapefruit → Chercher si effet indésirable ou toxicité
- * sauf *pro-drug* =inverse : exemple : *codéine*, *clopidogrel*
- NB : les métaboliseurs lents ou rapides des CYP450 peuvent avoir fonctionnellement le même effet que inhibiteurs/inducteurs

Interactions Pharmacodynamiques : (modification de l'effet sans modification de la concentration plasmatique →réfléchir en mécanisme d'action)

- 2molécules qui ont un effet net similaire: exemples :
 - un anticoagulant +un antiagrégant plaquettaire → saignement
 - IEC + spironolactone → hyperkaliémie
 - IEC + AINS → insuffisance rénale
 - BZD +antipsychotiques → sédation.....etc
- 2molécules qui ont un effet net inverse

Agents		Indications	Mechanism of action	Adverse effects	Contraindications	Interactions
St. John's Wort		Major depressive disorder	Downregulation of β-adrenergic receptors and inhibition of glutamate release ^[1]	- Gastrointestinal upset - Dizziness, headache, confusion	Use with caution in bipolar disorder ($\uparrow$ risk of provoking mania)	- Other serotonergic drugs: $\uparrow$ risk of serotonin syndrome - St John's Wort: induces cytochrome P450
Selective serotonin reuptake inhibitors (SSRIs; e.g., fluoxetine, sertraline, citalopram)		- Major depressive disorder (first-line: SSRIs) - Generalized anxiety disorder	Inhibition of serotonin reuptake in synaptic cleft $\rightarrow \uparrow$ serotonin levels	- Sexual dysfunction - Gastrointestinal upset - Agitation, insomnia, anxiety $\uparrow$ Blood pressure (SNRIs)	- Bleeding disorders - Epilepsy - SNRIs: uncontrolled hypertension	
Serotonin norepinephrine reuptake inhibitors (SNRIs; e.g., venlafaxine, duloxetine)		Neuropathic pain, stress incontinence, fibromyalgia (SNRIs)				
Tricyclic antidepressants (TCAs)	Secondary amines (e.g., nortriptyline, desipramine)	- Major depressive disorder - Neuropathic pain - Migraine prophylaxis	Inhibition of serotonin and norepinephrine reuptake in synaptic cleft $\rightarrow \uparrow$ serotonin and norepinephrine levels	- Anticholinergic effects - Cardiotoxicity: prolonged QT interval and widened QRS complex	Tertiary amines should be avoided in elderly individuals.	
	Tertiary amines (e.g., amitriptyline, imipramine)					
Serotonin antagonist and reuptake inhibitors (SARIs; e.g., trazodone)		- Insomnia - Major depressive disorder (high doses required)	Inhibition of H_1, α_1, and postsynaptic 5-HT₂ receptors as well as serotonin reuptake $\rightarrow \uparrow$ serotonin levels	- Priapism - Sedation	Avoid in individuals with risk factors for prolonged QT interval. ^[2]	
Monoamine oxidase inhibitors (MAOIs; e.g., tranylcypromine, phenelzine, selegiline)		- Major depressive disorder (especially atypical depression) - Parkinson disease (selegiline)	- Nonselective inhibition of monoamine oxidase $\rightarrow \uparrow$ levels of epinephrine, norepinephrine, serotonin, and dopamine - Selegiline: selective MAO-B inhibitor $\rightarrow \uparrow$ levels of dopamine	- CNS stimulation - Tyramine-induced hypertensive crisis	Pheochromocytoma ^[3]	
Atypical antidepressants	Mirtazapine	- Major depressive disorder (mirtazapine: especially individuals who are underweight and/or have insomnia) - Smoking cessation (bupropion, varenicline)	α_2, H_1, 5-HT₂, and 5-HT₃ receptor antagonists $\rightarrow \uparrow$ serotonin and norepinephrine release and $\uparrow$ effect of serotonin on free 5-HT₁ receptors	- Increased appetite, weight gain, and serum cholesterol and triglyceride levels - Sedation	Hypersensitivity ^[4]	
	Bupropion ^[5]		Thought to increase dopamine and norepinephrine levels via reuptake inhibition	- Reduction of seizure threshold - Insomnia, agitation, tachycardia	- Seizure disorder - Anorexia nervosa, bulimia nervosa	
	Vilazodone		- Inhibition of serotonin reuptake in synaptic cleft $\rightarrow \uparrow$ serotonin levels 5-HT_{1A} receptor partial agonist	- Sexual dysfunction - Sleep disturbance, abnormal dreams - Gastrointestinal upset	Concomitant MAOI use ^[6]	
	Vortioxetine		- Inhibition of 5-HT₃ receptors and serotonin reuptake in synaptic cleft $\rightarrow \uparrow$ serotonin levels 5-HT_{1A} receptor agonist			
	Varenicline		Nicotinic ACh receptor partial agonist	Sleep disturbances	Hypersensitivity	Transdermal nicotine: $\uparrow$ adverse events
	Buspirone	Anxiety disorder	5-HT_{1A} receptor stimulation	- Headaches, dizziness, nausea, - Seizures resulting from chronic use	Erythromycin, itraconazole, nefazodone: $\uparrow\uparrow$ plasma buspirone concentration ^[7]	